

Losing Lyumjev: changes since publication

Each change below gives where it goes in the live post and the text as it reads. The WordPress block HTML for each change, in the same order and headed by the same instruction, is in `changes_since_publication_wordpress.html`; paste a block into the code editor (Options, Code editor) at the place given. The full regenerated post is `diabettech-ultra-rapid-article-wordpress.html` in the same folder, if replacing the whole body is easier. Generated by `additions_since_publication.py` from the master article; the published version is commit `b6d7a42` with the two edits to the stand paragraph already made in WordPress.

1. New subsection: "The insulin that got through"

Where: Insert after the paragraph ending "on, makes it harder to prescribe." and before the heading "Cost before biosimilars".

Text:

The insulin that got through

Tresiba (insulin degludec), Novo Nordisk's ultra-long-acting insulin, is a useful comparison: a newer insulin that the same formularies did make room for. It also costs more than the insulins it would replace: £3.11 per 100 units at list price in English primary care in the year to July 2026, against £2.32 for Lantus and £2.00 for Semglee, the cheapest glargine biosimilar, which has been dispensed in England since 2019 ([prices](#)). Formularies made room for Tresiba with a cheaper long-acting insulin to protect. The ultra-rapid insulins were in that position only after Trurapi arrived in 2021, and before then they cost no more than the insulins they would replace.

I read the same 38 formularies for Tresiba, using the same documents and the same four categories as for Lyumjev. Tresiba is on 37 of them and Lyumjev on 25. Of the 13 areas that leave Lyumjev off, 12 list Tresiba, Northern Ireland and seven Scottish boards among them. Tresiba is the less restricted of the two in 20 areas and Lyumjev in 3; in the other 15 they are treated alike ([table](#)). Tresiba is seldom open to any prescriber either. Most formularies tie it to a specialist or to named uses, such as night-time hypoglycaemia or a carer giving the injection. What it has, almost everywhere, is a written place.

Part of the difference is process. As a new molecule, Tresiba had to be appraised. The [Scottish Medicines Consortium](#) accepted it for use in NHS Scotland in August 2016, and the All Wales Medicines Strategy Group recommended it in 2016 and again in 2022, so every board had a decision to adopt. Lyumjev, as a same-priced version of an existing insulin, was spared appraisal and was given no decision at all.

Part is the kind of evidence each had. Tresiba was tested in trials whose main outcome was hypoglycaemia: in [SWITCH 1](#), people with type 1 diabetes had fewer episodes on degludec than on glargine. When NICE revised its type 1 guidance on long-acting insulin in 2021, it named degludec for people with "a particular concern about nocturnal hypoglycaemia". Its mealtime recommendations still date from 2015 and name no product. A committee could point to a problem Tresiba addressed and a group of people to give it to. For the ultra-rapid insulins the headline was no difference in HbA1c, and the problem they address, waiting before a meal, is not one formularies record.

The dispensing follows a similar course. Counted from launch, Tresiba was taken up in England no faster than Fiasp for five years: 7.8% of long-acting analogue units at five years, against 7.9% of rapid-acting units for Fiasp. They separated after the 2017 hypoglycaemia trials, and at nine years Tresiba was at 19.6% and Fiasp at 13.9%. Areas that use more Tresiba and Toujeo also tend to use more ultra-rapid insulin (Spearman 0.49, 95% interval 0.25 to 0.67, across 59 UK areas), so local habit plays a part. Tresiba still reached places the faster mealtime insulins did not. Most long-acting insulin in primary care goes to people with type 2 diabetes, so these are not the same patients, and the timing fits the trials without showing that they caused the rise.

Abroad the route differs, and appraisal can shut a door as well as open one. France's assessment committee re-rated Tresiba in 2019 as a minor improvement over glargine (ASMR IV) for people at high risk of hypoglycaemia, on the strength of those trials, while it rated [Fiasp](#) and [Lyumjev](#) as no improvement (ASMR V). In Germany every benefit assessment of degludec found no added benefit, and [Novo Nordisk took Tresiba off the market](#) from January 2016 until December 2018. Australia's [PBAC rejected it](#) in 2013, and it was subsidised only from July 2026, while Fiasp was listed there by the end of 2019 at the same price as NovoRapid. Outside Australia, wherever both can be measured, Tresiba now takes a larger share of long-acting insulin than the ultra-rapid insulins take of rapid-acting: 22% against 15% in France in 2025, 41% against 11% in Denmark, and 20% against 9% in the US Medicare drug programme in 2024 ([figures](#)). Price parity for the faster insulins was also a mostly British arrangement. In France Fiasp is listed at €2.09 per 100 units against €1.48 for NovoRapid, and in Denmark at 32.1 kroner against 16.7. Denmark's reimbursement committee counted Fiasp among "de dyrere lægemidler" (the more expensive medicines) in 2019, and since 2022 it has been reimbursed only for people whose after-meal glucose is not managed on another rapid insulin.

2. Replacement paragraph

Where: Replace the paragraph "From 2021 there was a cheaper insulin to"… with the text below. It comes straight after the heading "Cost after biosimilars".

Text:

From 2021 there was a cheaper insulin to protect. Trurapi and the other biosimilars are about 30% cheaper than NovoRapid and work at the same speed. [Norfolk and Waveney](#) estimated that moving 80% of people from NovoRapid to Trurapi would save about £340,000 a year, and [Cheshire and Merseyside](#) about £1 million. Each is about an eighth of what the area now spends on rapid-acting analogues at list price. NHS Dorset's [2026/27 prescribing incentive scheme](#) asks primary care networks to make biosimilar aspart at least half of all aspart prescribing. Those documents exclude Fiasp from the switch, but once Trurapi is first choice an ultra-rapid insulin becomes a second step, and the trial evidence gives that step little to stand on. Trurapi reached 7.5% of units within five years. Lyumjev, after six, is at 3.7%.

3. New paragraph

Where: Insert after the paragraph ending "in each of these markets (figures)." and before the paragraph "For context, Lilly's total revenue in the first".

Text:

Pricing every market at the NHS list price, the figure the BNF prints, puts them on one scale. On that basis the UK's rapid-acting analogue market came to £174 million over the latest twelve months: £143 million in England, £16 million in Scotland, £9.1 million in Wales and £6.0 million in Northern Ireland. Lyumjev was £7.1 million of it (4.1%) and Fiasp £24.6 million. France's 2025 community dispensing would cost £170 million at the same prices, with Lyumjev £6.1 million of it. Germany's statutory-insurance market would cost £218 million, and there Lyumjev is pooled with Liprolog in a line worth £34 million. Local prices differ from the NHS's, so these are volumes priced in pounds and say nothing about what each country paid ([figures](#)).

4. New paragraphs

Where: Insert after the paragraph ending "while the UK still has Lyumjev." and before the heading "The limits".

Text:

What I want to see now is the diabetes charities and patient organisations of Europe standing up and being counted, and so far they have barely made a sound. Diabetes UK's page on insulin supply lists ten discontinuations and shortages and does not mention Lyumjev or the withdrawals across Europe, and Breakthrough T1D UK's site mentions Lyumjev only in its guide to connected pens. In France, which has already lost the cartridges, the last thing I can find on Lyumjev from the national federation of people with diabetes is a 2023 note about a temporary shortage. Germany's diabetesDE has nothing on it, and the German Diabetes Society's statement was practical advice on pump cartridge shortages ([search record](#)). Norway's diabetes association reported the withdrawals and quoted a professor reassuring readers that there were good alternatives to every insulin going. diabetesschweiz complained to Lilly, then passed on Lilly's explanation that hardly anyone used it. Reporting a withdrawal and relaying the manufacturer's reasons is the least an organisation can do, and these organisations exist to speak for the people who use these insulins. They should be demanding that the faster insulins stay on the market, that they are part of the fully closed-loop systems now in trials, that formularies stop parking them behind the insulin people are already on, and that clinicians offer them to everyone who might benefit and let each person decide for themselves whether to turn them down. When they stay quiet, the manufacturer's account of low demand is the only one anyone hears.

Is this a failure of the system? Yes, at every level, and nobody escapes the blame. The licensing trials were built to show the new insulins were no worse on HbA1c, and that was read as showing they were no better. NICE left its mealtime insulin advice as it stood in 2015, looked at it again in January 2026 and left it there. The Scottish and Welsh appraisal bodies wrote rules that left Lyumjev without a decision in either country, and Northern Ireland, which depends on them, inherited the gap. Formulary committees left it off or put it behind the insulin people were already using, while finding a written place for Tresiba, which costs a third more than Lantus. Consultants told people who asked for a faster insulin that they were too well controlled to need one. Lilly priced Lyumjev the same as Humalog and then, as far as the public record shows, did almost nothing to make the case for it in the UK, and did not even tell the Swiss patient organisation that it was going. The charities, whose whole purpose is to make noise about exactly this, have barely raised their voices. The cost of all of it falls on the people with diabetes who were never offered the choice.

5. Replacement paragraph

Where: Replace the paragraph "The method, the code, the formulary wording area"... with the text below. It comes straight after the paragraph ending "how common any of it is".

Text:

The method, the code, the formulary wording area by area and the outputs behind every number are at <https://github.com/tim2000s/rapid-insulin-prescribing>. The sources follow.

6. New: sources at the foot

Where: Add at the very end of the post, after the closing paragraph "...The sources follow.": a heading "Sources" and the numbered list (67 entries).

Text:

Sources

Sources for "Losing Lyumjev: how the system let its patients down", in approximately the order the article draws on them. 1. NHS Business Services Authority. English Prescribing Dataset and English Prescribing Dataset with SNOMED code; Hospital prescribing dispensed in the community; Secondary Care Medicines Data with indicative price (finalised). <https://opendata.nhsbsa.net> (accessed 30 September 2026) 2. Public Health Scotland. Prescriptions in the community. <https://www.opendata.nhs.scot/dataset/prescriptions-in-the-community>; Business Services Organisation Northern Ireland. GP prescribing data. <https://www.opendatani.gov.uk/@business-services-organisation/gp-prescribing-data>; NHS Wales Shared Services Partnership. General practice prescribing data extract. <https://nwssp.nhs.wales> (all accessed 30 September 2026) 3. Assurance Maladie. Open Medic: base complète sur les dépenses de médicaments interrégimes; Medic'AM. <https://open-data-assurance-maladie.ameli.fr/medicaments/> (accessed 30 September 2026) 4. Sundhedsdatastyrelsen. medstat.dk: data download. <https://medstat.dk/en/download> (accessed 30 September 2026) 5. GKV-Spitzenverband. GKV-Arzneimittel-Schnellinformation (GAmSi), Bundesberichte 2020 to 2026. <https://www.gkv-gamsi.de> (accessed 30 September 2026) 6. Australian Government Department of Health. PBS date of supply data. <https://www.pbs.gov.au/info/statistics/dos-and-dop/dos-and-dop> (accessed 30 September 2026) 7. Centers for Medicare and Medicaid Services. Medicare Part D spending by drug; Medicare Part D prescribers by geography and drug. <https://data.cms.gov> (accessed 30 September 2026) 8. Centers for Medicare and Medicaid Services. State Drug Utilization Data. <https://data.medicare.gov> (accessed 30 September 2026) 9. NHS Business Services Authority. Dictionary of medicines and devices browser: actual medicinal product packs for Fiasp, NovoRapid, Lyumjev, Humalog, Trurapi and Admelog. <https://dmd-browser.nhsbsa.nhs.uk> (data as of 24 September 2026, accessed 30 September 2026) 10. European Medicines Agency. Eli Lilly insulin (various forms): supply shortage. First published 23 March 2026; and medicine shortage communication on the marketing cessation of selected presentations of Lilly insulins, adopted 16 March 2026. <https://www.ema.europa.eu/en/medicines/human/shortages/eli-lilly-insulin-various-forms-supply-shortage> (accessed 1 October 2026) 11. European Medicines Agency. Fiasp PumpCart (insulin aspart): supply shortage. First published 12 May 2025. <https://www.ema.europa.eu/en/medicines/human/shortages/fiasp-pumpcart-insulin-aspart> (accessed 1 October 2026) 12.

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